

1. Administrative & Study Identification

Full Study Title: A Phase III, Randomized, Placebo-controlled, Double-blind, Multi-center, International Study of Durvalumab With Stereotactic Body Radiation Therapy (SBRT) for the Treatment of Patients With Unresected Stage I/II, Lymph-node Negative Non-small Cell Lung Cancer (PACIFIC-4/RTOG-3515) Osimertinib Following SBRT, a Single Arm Cohort for Patients With Unresected Stage I/II, Lymph Node Negative NSCLC Harboring a Sensitizing EGFR Mutation **Short Title/Acronym:** PACIFIC-4 / RTOG-3515 **Protocol Number:** D9103C00001 **NCT Number:** NCT03833154 **Sponsor Name:** AstraZeneca **Investigational Product:** Durvalumab (Imfinzi) [ATC: L01FF03] / Osimertinib **Phase of Development:** Phase III **Medical Monitor:** AstraZeneca Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To assess the efficacy of Durvalumab combined with standard of care (SoC) Stereotactic Body Radiation Therapy (SBRT) compared to placebo with SoC SBRT in terms of Progression-Free Survival (PFS) in the main cohort, and to assess 4-year PFS for Osimertinib following SBRT in the EGFR-mutated cohort. **Secondary Objectives:** To evaluate Overall Survival (OS), Time to Progression (TTP), pharmacokinetics (PK) including concentration of durvalumab in serum (peak concentration and trough), immunogenicity, and Health-Related Quality of Life (HRQoL).

2.2 Background & Rationale To address the gap in adjuvant treatment for patients with early-stage, medically inoperable or unresected NSCLC. SBRT is the standard of care for patients with unresectable stage I/II, lymph-node negative NSCLC, yielding high rates of primary tumor control. However, regional and distant failure remains high. Durvalumab has demonstrated significant survival benefits in stage III unresectable NSCLC (the PACIFIC regimen). This study investigates whether administering durvalumab concurrently with and following SBRT can improve progression-free and overall survival in early-stage NSCLC.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Placebo-controlled (for the main cohort); Single-arm (for the EGFRm Osimertinib cohort) **Blinding:** Double-blind (for the main cohort) **Randomization:** 1:1 (Main cohort: Durvalumab + SBRT vs. Placebo + SBRT)

3.2 Planned Enrollment & Duration Target \$N\$: 724 participants
Trial Status: Active, Not Recruiting **Number of Sites:** Multi-center, International **Estimated Study Start:** 05-Mar-2019
Estimated Primary Completion: 04-Apr-2028

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age ≥ 18 years with a WHO/ECOG Performance Status of 0, 1, or 2. 2. Unresected clinical Stage I/II lymph node-negative (T1 to T3N0M0) NSCLC with planned SoC SBRT as definitive treatment. 3. Life expectancy of at least 12 weeks, body weight > 30 kg, and adequate organ and marrow function.

4.2 Exclusion Criteria 1. Preoperative or adjuvant platinum-based or other chemotherapy for the disease under investigation, or prior treatment with neoadjuvant/adjuvant EGFR TKI. 2. Past medical history of Interstitial Lung Disease (ILD), drug-induced ILD, or radiation pneumonitis requiring steroid treatment. 3. Mean resting corrected QT interval > 470 msec, clinically important abnormalities in resting ECG, or current use of potent CYP3A4 inducers.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Durvalumab 1500 mg administered every 4 weeks for up to 26 cycles (approx. 24 months), concurrent with SoC SBRT (delivered in 3, 4, 5, or 8 fractions). An independent cohort of EGFR-mutated patients will receive targeted therapy (Osimertinib) following SBRT. **Route of Administration:** Intravenous (IV) for Durvalumab/Placebo; Oral for Osimertinib. **Duration of Treatment:** Up to 24 months for the immunotherapy phase.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care medications. **Prohibited:** Potent inducers of CYP3A4 (particularly for the Osimertinib cohort) and any concurrent investigational anticancer therapies.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Up to -70 Days	Informed Consent, PET-CT Staging (within 10 weeks), ECG, Tumor Tissue Submission
Baseline	Day 0	Randomization, First IV Dose, Initiation of SoC SBRT
Interim	Every 4 Weeks	IV administration (Durvalumab/Placebo), Safety Labs, HRQoL (EORTC QLQ-C30)
End of Study	Up to 24 Months	Final PFS assessment (PFS24), OS Follow-up, ADA neutralizing antibody detection

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: 1. Progression-Free Survival (PFS) assessed by Blinded Independent Central Review (BICR) according to RECIST 1.1 in a subpopulation of patients with Stage I/II NSCLC (Main Cohort). 2. 4-year Progression-Free Survival (4y-PFS) by ICR according to RECIST 1.1 criteria (Osimertinib Cohort).

7.2 Secondary & Exploratory Endpoints 1. Progression-Free Survival (PFS) assessed by BICR per RECIST 1.1 in all randomized patients with Stage I/II NSCLC. 2. Overall Survival (OS). 3. Health-related quality of life (EORTC QLQ-C30) and concentration of durvalumab in serum such as peak concentration and trough.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard clinical oncology evaluation, continuous tracking of immune-mediated adverse events (imAEs) and SBRT-related toxicities (e.g., radiation pneumonitis). **Serious Adverse Events (SAEs):** Reported within 24 hours to the Sponsor and Institutional Review Boards. **Data Monitoring Committee (DMC):** Independent Data Monitoring Committee (IDMC) utilized to ensure patient safety and evaluate risk/benefit ratios throughout the trial.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) for primary efficacy (PFS/OS). Safety analysis set for all patients receiving at least one dose of study treatment. **Sample Size Justification:** Target $N=724$, appropriately powered to detect a statistically significant improvement in PFS for the Durvalumab + SBRT arm and assess efficacy in the Osimertinib arm. **Handling of Missing Data:** Standard oncology censoring rules applied for PFS; patients without documented progression censored at the date of last evaluable tumor assessment.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Routine onsite and remote monitoring for protocol adherence, source data verification, and RECIST 1.1 criteria compliance. **Audit Trail:** Validated Electronic Data Capture (EDC) systems with full audit trails and data clarification processes.

11. Study Identifiers & Governance

Posting ID: {{668951216641878848}} **Primary ID:** {{D9103C00001}}
Registry Number: {{NCT03833154}} **Sponsor/Lead Organization:** {{AstraZeneca}} **Study Title:** {{Durvalumab With Stereotactic Body Radiation Therapy (SBRT) vs Placebo With SBRT in Early Stage Unresected Non-small Cell Lung Cancer (NSCLC) Patients/ Osimertinib Following SBRT in Patients With Early Stage Unresected NSCLC Harboring an EGFR Mutation}} **Official Regulatory Title:** {{A Phase III, Randomized, Placebo-controlled, Double-blind, Multi-center, International Study of Durvalumab With Stereotactic Body Radiation Therapy (SBRT) for the Treatment of Patients With Unresected Stage I/II, Lymph-node Negative Non-small Cell Lung Cancer (PACIFIC-4/RTOG-3515) Osimertinib Following SBRT, a Single Arm Cohort for Patients With Unresected Stage I/II, Lymph Node Negative NSCLC Harboring a Sensitizing EGFR Mutation}}

12. Clinical Framework (Oncology/NSCLC Specific)

Primary Condition: {{Carcinoma, Non-Small-Cell Lung (Early Stage Unresected NSCLC)}} **Diagnosis Threshold:** {{Stage I/II lymph node-negative (T1 to T3N0M0) NSCLC}} **Medical Methodology:** {{Stereotactic Body Radiation Therapy (SBRT) combined with immune checkpoint blockade}} **Optimization Target:** {{Progression-Free Survival (PFS) and Overall Survival (OS)}}

13. Study Procedures & Methodology

Management Pathway: {{Standard of Care SBRT followed by adjuvant targeted/immunotherapy}} **Education Protocol (HCP):** {{Protocol-specific administration, RECIST 1.1 staging, and imAE management training}} **Education Protocol (Patient):** {{Informed consent, SBRT procedural guidelines, and toxicity reporting}} **Quality Audit Mechanism:** {{Blinded Independent Central Review (BICR) for tumor response assessments}}

14. Observation & Follow-Up Schedule

Total Duration: {{Up to 26 cycles (approx. 24 months) of treatment, plus long-term OS follow-up}} **Onsite Visit Cadence:** {{Every 4 weeks (Q4W) for IV drug administration}} **Remote Monitoring Frequency:** {{As per standard oncology trial protocol for survival follow-up}} **Checklist Review Interval:** {{Prior to each 4-week dosing cycle}}

15. Sign-off & Approval

Role	Name	Signature	Date		:---	:---	:---	:---
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]					
Statistician	[Name]	_____	[DD-MMM-YYYY]					